YG Critical Care Unit

Surgical Tracheostomy Checklist

ICU Consultant	
Surgical Consultant	
Anaesthetic Consultant	

Addressograph

Name

Hospital Number

Date of Birth

Booking the PROCEDURE

Rational for surgical tracheostomy:

Assessment of risk grade:

Routine or Elective	High Risk or Emergent
• No anticipated complications • Low level respiratory support • Grade 1/2 upper airway • Could be a candidate for perc. Tracheostomy	• Complex - Anatomy/CVS/RS/Coag • PEEP >5 / PS >10 / FiO2 >40% / leak / Hx of desats on rolls • Difficult upper airway • Not a candidate for perc Tracheostomy

Senior presence +/- 2nd Anaesthetist should be strongly considered for all ITU surgical tracheostomies

NOK informed ☐

Appropriate Consent [eg Form 4] ☐

Medicines and coagulation checked ☐ Group and save sent ☐ Covid Status Recheck ☐

Tracheostomy size/type requested Confirmed available ☐

Procedure booked for

Feed to stop at

Confirmed with Surgical team ☐ Confirmed with Anaesthetic on call ☐

Before moving to theatre

Intra hospital transfer sheet used ☐

Tracheostomy tube with pt ☐

Handover of reasons/risk and planned procedure ☐

Hand over on return

Type of incision and tracheostomy sited

Position confirmed with bronchoscope: tip is _____cm from carina ☐

Tracheostomy Chart completed at bedside, blue 'tracheostomy emergences' box in place ☐

Established on ventilator: settings reviewed, capnography in situ, no leak, HME removed, correct closed suction ☐

Cuff Pressure recorded: _____ mmHg (inform senior if >30mmHg) ☐

Sedation reviewed; timing of drugs (local anaesthetic & NMB) documented ☐

Surgical notes and anaesthetic chart available ☐

Adverse events documented in notes and Datix submitted (or N/A) ☐

Post-procedure handover to nursing staff ☐

Chest x-ray ordered ☐ Seen ☐ Report Seen ☐

Signed by responsible practitioner:

Print: GMC/NMC:

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Anaesthetics Department
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